

Women's Specialists of Clear Lake

Dr. Geoffrey H. Klein, MD · FACOG · DABOM · 400 W Medical Center Blvd, Suite 300, Webster, TX 77598 · (281) 557-0300



Group B Streptococcus (GBS)

Testing, prevention, and protecting your newborn during labor and delivery

WHAT IS GBS?

Group B Streptococcus (GBS) is a type of bacteria naturally found in the digestive and genital tract. About **10–30% of pregnant women** carry GBS. It causes no illness in the mother but can be passed to the baby during birth and cause serious infection. It is NOT a sexually transmitted infection.

HOW & WHEN YOU'RE TESTED

- Routine vaginal-rectal swab at **35–37 weeks**
- Results in 1–5 days
- Carrier status can change — test each pregnancy
- If GBS positive in urine during pregnancy — treat & assume positive for delivery
- If GBS status unknown at delivery — treat based on risk factors

TREATMENT DURING LABOR

- **IV Penicillin G** — first-line antibiotic **RX**
- Given every 4 hours during labor until delivery
- **Ampicillin** — alternative **RX**
- **Cefazolin** — for penicillin allergy (low-risk) **RX**
- **Clindamycin / Vancomycin** — severe penicillin allergy **RX**
- Antibiotics must be given at least 4 hours before delivery to be most effective

WHO NEEDS ANTIBIOTICS IN LABOR

- GBS-positive culture result at 35–37 weeks
- GBS bacteriuria (in urine) during this pregnancy
- Previous baby with GBS disease
- GBS status unknown AND any of the following:
 - Delivery <37 weeks gestation
 - Water broken ≥18 hours before delivery
 - Fever during labor ≥100.4°F

GBS IN NEWBORNS

- **Early-onset:** 0–6 days of life; pneumonia, meningitis, sepsis
- **Late-onset:** 7 days–3 months; bloodstream or meningitis
- Risk to baby reduced by 80%+ with intrapartum antibiotics
- Babies observed closely for 24–48 hours after birth
- Most GBS-exposed babies remain healthy

Without treatment: ~1–2% of GBS+ mothers have infected babies. With treatment: risk drops to ~0.1%.

GBS IN PREGNANCY (RARE)

- GBS can occasionally cause UTI during pregnancy
- Treated with oral antibiotics (amoxicillin, nitrofurantoin) **RX**
- Rarely causes chorioamnionitis (uterine infection)
- Not necessary to treat vaginal carrier state before labor
- Preterm labor may prompt early treatment decisions

⚠️ WATCH YOUR NEWBORN FOR THESE SIGNS OF GBS INFECTION

- ⚠️ Fever or low temperature in newborn
- ⚠️ Grunting, fast breathing, or difficulty breathing
- ⚠️ Limpness or stiffness
- ⚠️ Bulging fontanelle (soft spot on head)
- ⚠️ Poor feeding or extreme sleepiness
- ⚠️ Pale, bluish, or blotchy skin color
- ⚠️ Irritability or high-pitched cry
- ⚠️ Seizures — go to ER immediately

♥️ A Positive GBS Test Is Common — Not Dangerous with Proper Care

Testing positive for GBS is not a reflection of hygiene or sexual history. It is simply part of your body's normal bacterial environment. With IV antibiotics given during labor, the risk to your baby is dramatically reduced and most GBS-positive mothers have perfectly healthy newborns.